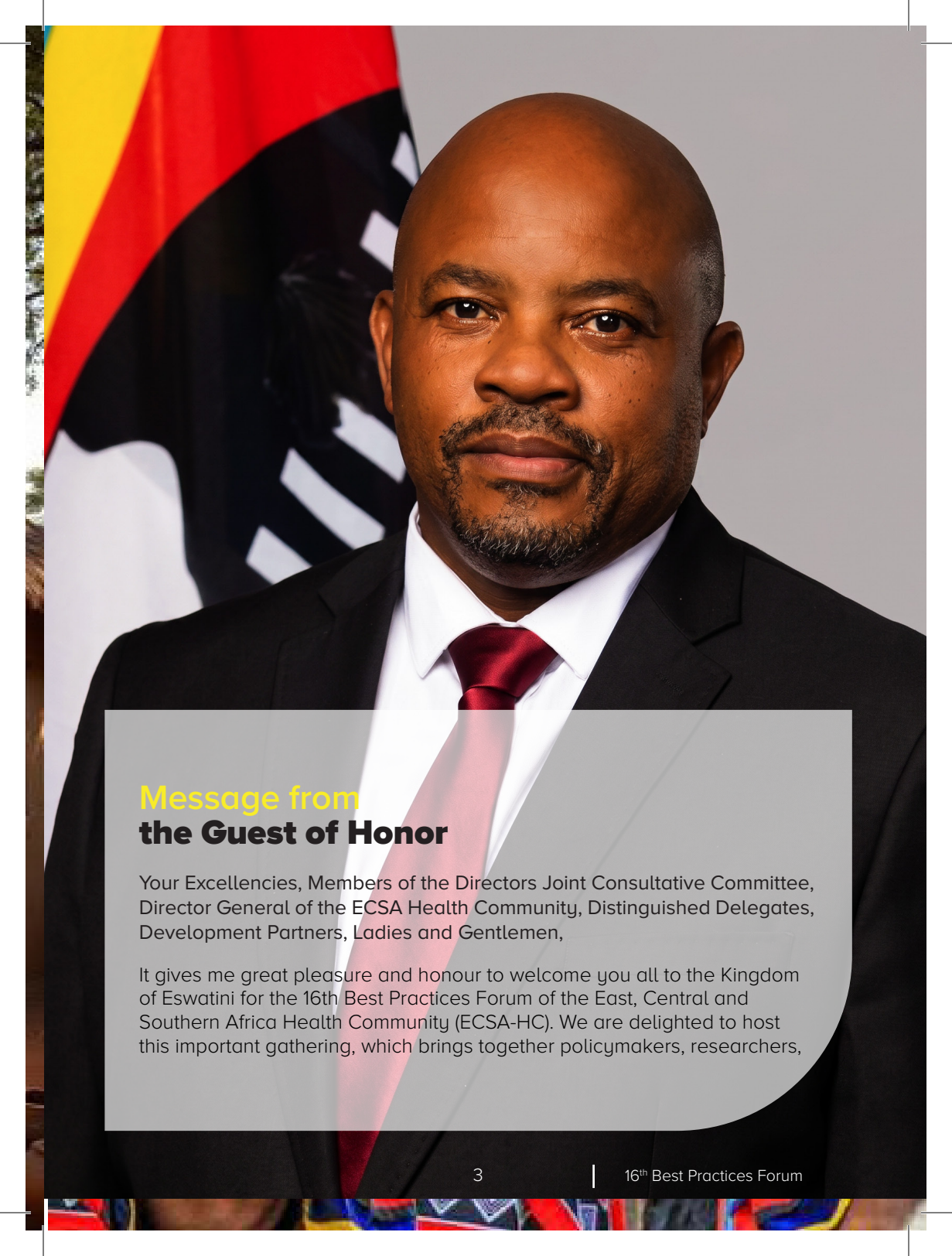


A portrait of a middle-aged Black man with a shaved head and a short beard, wearing a dark suit, white shirt, and a red tie. He is looking directly at the camera with a slight smile. In the background, the flag of Eswatini is visible, featuring horizontal stripes of red, yellow, and black, with a white isihlangu (shield) in the center. The text is overlaid on a semi-transparent grey box in the lower half of the image.

Message from the Guest of Honor

Your Excellencies, Members of the Directors Joint Consultative Committee, Director General of the ECSA Health Community, Distinguished Delegates, Development Partners, Ladies and Gentlemen,

It gives me great pleasure and honour to welcome you all to the Kingdom of Eswatini for the 16th Best Practices Forum of the East, Central and Southern Africa Health Community (ECSA-HC). We are delighted to host this important gathering, which brings together policymakers, researchers,

health professionals, development partners, and innovators from across the region to exchange experiences, showcase solutions, and strengthen collaboration in advancing health outcomes for our people.

***“Shaping the Future of Health in the ECSA-HC Region:
Advancing Health Security, Innovation and Sustainability”***

The theme of this year’s Forum is both timely and relevant. Our region continues to face complex and evolving challenges, including emerging disease threats, climate-related health risks, health workforce shortages, and increasing financial pressures on health systems. These challenges call for innovative approaches, stronger partnerships, and sustainable investments that can safeguard the health and well-being of our populations.

Eswatini remains committed to strengthening its health system through investments in primary health care, disease prevention and control, digital health, workforce development, and health security. However, we recognize that many of the challenges we face transcend national borders and require collective regional action. Through collaboration and shared learning, we can accelerate progress towards Universal Health Coverage, health security, and sustainable development.

This Forum provides an invaluable opportunity to learn from one another, identify scalable best practices, and build partnerships that translate knowledge into action. I encourage all participants to engage actively, share openly, and use this platform to develop practical solutions that can be adapted and implemented across our Member States.

Let me also express our sincere appreciation to ECSA-HC for its continued leadership in fostering regional cooperation, knowledge exchange, and collective action in health.

Once again, I warmly welcome you to the Kingdom of Eswatini and wish you productive discussions, meaningful networking, and a successful Forum.

Honourable **Mduduzi Matsebula (MP)**
Minister of Health, The Kingdom of Eswatini



About Kingdom of Eswatini

Eswatini, formerly known as Swaziland, is a small, landlocked kingdom in Southern Africa bordered by South Africa to the west and Mozambique to the east. With a population of approximately 1.2 million people, Eswatini is known for its rich cultural heritage, peaceful society, and strong traditional governance led by the monarchy. The country blends modern state institutions with longstanding cultural practices, creating a unique governance system that integrates both tradition and contemporary law. Eswatini's landscape ranges from the mountainous Highveld to the subtropical Lowveld, making it one of Africa's most scenic nations.

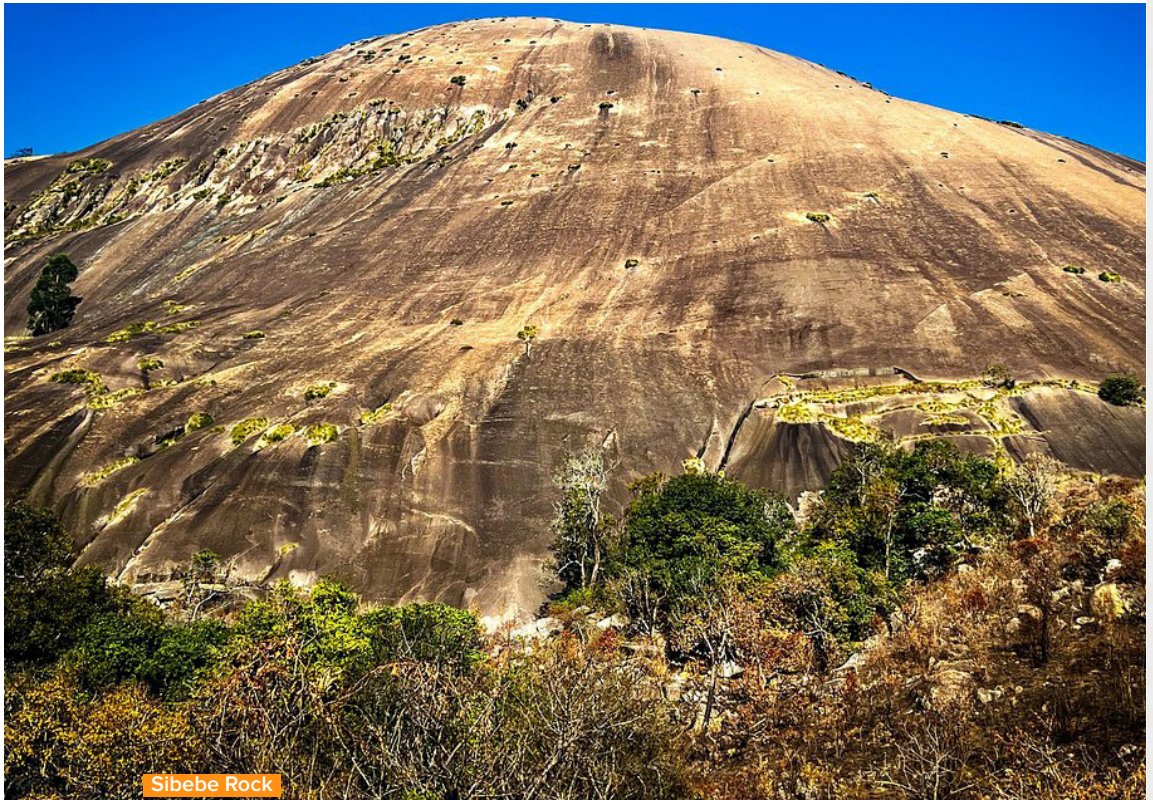
Eswatini's economy includes agriculture, manufacturing, mining, and services. Sugar is still a major export, along with citrus crops, textiles, forestry, and soft drink concentrates. The country benefits from its strategic location, integrated markets, and SACU infrastructure. Cultural festivals like the Umhlanga (Reed Dance) and Incwala Ceremony, game reserves, and Mlilwane and Hlane Royal National Parks are driving tourism in Eswatini. Eswatini spends heavily on health and well-being. Regional hospitals, clinics, public health units, and community-based services make up the Ministry of Health's health system. The Mbabane Government Hospital, Raleigh Fitkin Memorial Hospital, and several speciality centres are notable. The country is expanding its infrastructure, manpower, and service delivery capacity for steeply subsidised public healthcare. Mission hospitals and the private sector complement each other.

Eswatini has made major strides in addressing communicable diseases, particularly HIV/AIDS. Once having one of the highest HIV prevalence rates globally, the country has achieved remarkable progress through widespread antiretroviral therapy (ART), Prevention of Mother-to-Child Transmission (PMTCT) programs, and robust community engagement. HIV incidence and AIDS-related mortality have both significantly declined, demonstrating strong health system resilience and international collaboration. Tuberculosis control has also improved through integrated HIV–TB strategies.

As in other countries in the region, Eswatini has a rising rate of non-communicable diseases (NCDs) such as hypertension, diabetes, cardiovascular disease, and cancer. Lifestyle changes, diet risks, and urbanisation cause these tendencies.

Depression, substance use, and youth psychosocial disorders are becoming national priorities, but resources are scarce and stigmatised. The country is vulnerable to developing infectious illnesses, natural disasters, and climate change, emphasizing the need for better planning and response.

Recently, Eswatini has made significant health system changes. These include expanding NCD screening programs, community health workers in primary care, and emergency medical services. Electronic logistics management and patient information systems are being implemented to improve efficiency and data-driven decision-making. Eswatini coordinated rapid testing expansion, risk communication, and vaccination deployment during the COVID-19 pandemic. Universal health coverage (UHC), climate-resilient health infrastructure, workforce training, and maternal, neonatal, and adolescent health care are important priorities. To accelerate health sector transformation and offer equitable, high-quality treatment, Eswatini works with regional bodies like ECSA-HC, SADC, WHO AFRO, and development partners.

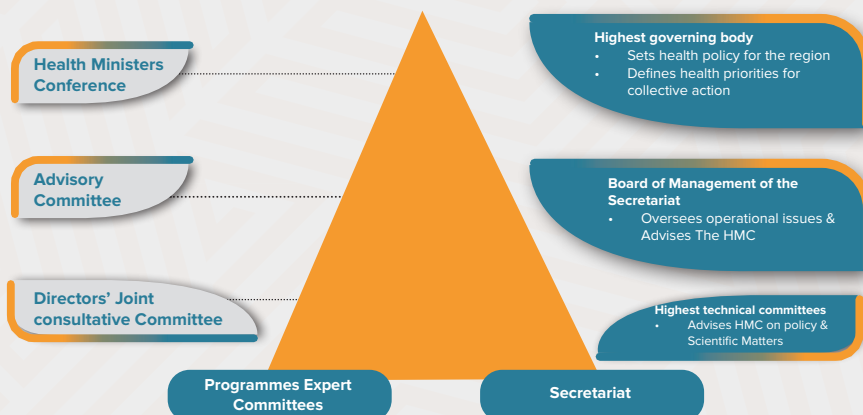


Sibebe Rock

About ECSA-HC

The East, Central and Southern Africa Health Community (ECSA-HC) is a regional intergovernmental organization dedicated to improving health outcomes across its member states: Eswatini, Kenya, Lesotho, Malawi, Mauritius, Mozambique, Sao Tome & Principe, Tanzania, Uganda, Zambia, and Zimbabwe. Recognizing that the diseases know no borders, ECSA-HC has also been working collaboratively with other neighboring countries to advance the health agenda in the region. The organization was established in 1974 under the auspices of Commonwealth Regional Health Secretariat. The organization has been advancing regional support to the member states.

Governance structure of ECSA-HC

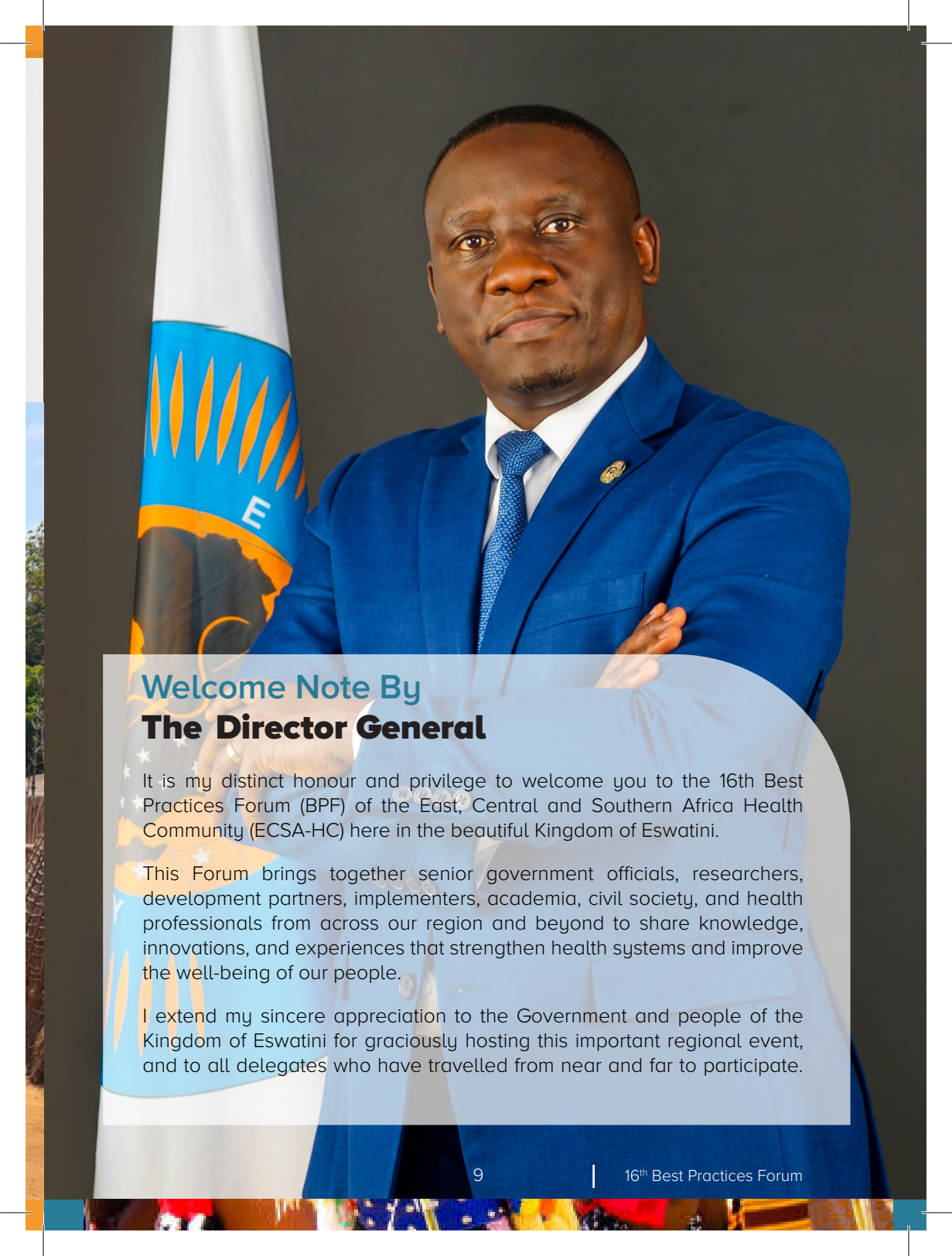


The East, Central and Southern Africa Health Community (ECSA-HC) is governed by three key organs: the Health Ministers Conference (HMC), which provides strategic leadership and policy direction; the Advisory Committee of Permanent/Principal Secretaries, which serves as the Management Board; and the Directors Joint Consultative Committee (DJCC), which provides technical guidance. ECSA-HC supports Member States in policy harmonization, health workforce development, disease prevention and control, health systems strengthening, maternal and child health, NCDs, nutrition, research, and health information systems through four technical clusters: Health Systems and Capacity Development; Family Health and Infectious Diseases; NCDs, Food Security and Nutrition; and Knowledge Management, Monitoring and Evaluation.

ECSCA-HC collaborates with a range of partners, including regional organizations such as the East African Community (EAC), African Union (AU) and the Southern African Development Community (SADC); international donors like the Global Fund, World Bank, USAID, USCDC as well as collaborating partners such as the World Health Organization (WHO), Africa CDC, ASLM, Amref Health Africa, IGAD, Gates Foundation and other various development and implementing partners. These collaborations enhance the reach and impact of ECSCA-HC's health initiatives, ensuring sustainable health development and improved well-being across the region.



Mantenga Village



Welcome Note By The Director General

It is my distinct honour and privilege to welcome you to the 16th Best Practices Forum (BPF) of the East, Central and Southern Africa Health Community (ECSA-HC) here in the beautiful Kingdom of Eswatini.

This Forum brings together senior government officials, researchers, development partners, implementers, academia, civil society, and health professionals from across our region and beyond to share knowledge, innovations, and experiences that strengthen health systems and improve the well-being of our people.

I extend my sincere appreciation to the Government and people of the Kingdom of Eswatini for graciously hosting this important regional event, and to all delegates who have travelled from near and far to participate.

Your presence reflects our collective commitment to advancing health, resilience, and sustainable development across the ECSA region.

As our region faces emerging health threats, shifting financing landscapes, climate-related challenges, and growing demands for quality healthcare, the need for collaboration, innovation, and shared learning has never been greater.

*Under this year's theme, "**Shaping the Future of Health in the ECSA-HC Region: Advancing Health Security, Innovation and Sustainability**," we will:*

- ▶ Showcase evidence-based innovations and best practices that are improving health outcomes across our Member States;
- ▶ Explore solutions to strengthen health security, resilient health systems, workforce development, digital transformation, and sustainable financing;
- ▶ Promote South-South cooperation and regional partnerships that accelerate the scale-up of effective interventions; and
- ▶ Advance collaboration to address shared challenges and achieve Universal Health Coverage.

Over the next two days, I encourage you to engage actively, share experiences, forge partnerships, and identify practical solutions that can be translated into action across our countries and institutions.

The recommendations emerging from this Forum will be submitted to the Directors Joint Consultative Committee (DJCC) and subsequently presented to the 78th Health Ministers Conference (HMC) for consideration as regional policy directions and resolutions.

Together, let us reaffirm our vision of a healthier, more resilient, and prosperous region for all.

I wish you fruitful deliberations and a successful Forum.

Dr. Ntuli A. Kapologwe
Director General–ECSA HC



About the **ECSA-HC Best Practices Forum**

The Best Practices Forum (BPF) is a flagship annual event of the East, Central and Southern Africa Health Community (ECSA-HC), initiated in 2002 as a knowledge-sharing platform to promote evidence-based policy and programmatic decisions in the health sector. The forum provides a structured space for member states, technical experts, policymakers, researchers,

and partners to present and learn from innovative practices, successful interventions, and lessons across the region.

Over the years, the BPF has evolved into a robust platform influencing health policies and service delivery through the adoption of regional resolutions by the ECSA-HC Health Ministers Conference. It has catalyzed knowledge translation and peer learning, contributed to improved programming, and informed strategic investments in public health.

Each edition of the forum aligns its theme with regional and global health priorities, addressing emerging challenges and opportunities in health systems.

Sub-Themes and Tracks

Sub-theme 1. Regional Health Security & Pandemic Agreement Implementation Track 1.1: Operationalizing the Pandemic Agreement and IHR (2005) Reforms in ECSA region Track 1.2: Surveillance, Early Warning, and One Health Intelligence for preparedness and response Track 1.3 Prevention and Control of Antimicrobial Resistance Track 1.4 Workforce Capacity and Emergency Preparedness	Sub-theme 2. Digital Innovation for Resilient Health Systems (including AI) Track 2.1: Scaling Digital Health Solutions for Service Delivery and UHC Track 2.2: Artificial Intelligence and Data-Driven Decision-Making in Health Systems Track 2.3 Digital Innovation and Youth-Led Solutions in Health
Sub-theme 3. Harnessing Demographic Dividend & Youth Engagement	Sub-theme 4. Global Health Diplomacy Amid Reforms



Track 3.1: Youth-Centered Health Systems and Meaningful Engagement Track 3.2: Leveraging the Demographic Dividend for Health and Economic Development Track 3.3: Investing in Adolescent and Youth Sexual and Reproductive Health Track 3.4: Engaging Young Professionals in Leadership, Research, and Policy Development Track 3.5: Strengthening Adolescent Nutrition with Evidence-Based Interventions to Promote Healthy Youth Development	Track 4.1: Advancing Regional Voice and Negotiation Power in Global Health Governance Track 4.2: Strategic Positioning of ECSA-HC Priorities in Global Health Agendas
Sub theme 5. Sustainable Financing & Partnerships Track 5.1: Domestic Resource Mobilization and Innovative Health Financing Models Track 5.2: Transforming Partnerships for Sustainable and Resilient Health Systems	Sub theme 6. Innovation, Health Research & Development in ECSA Track 6.1: Innovations in RMNCAH and Priority Population Health Interventions Track 6.2: Advancing Health Research, Innovation Ecosystems and Knowledge Translation
Sub theme 7. Quality of Care & Patient Safety Track 7.1: Improving Quality of Care Across the Continuum of Services Track 7.2: Patient Safety Systems and Culture in Health Care	

Dr **More Mungati**
Director of Programs, ECSA - HC